

[27,28]

Trigger factors may be involved in the first manifestation of psoriasis or contribute to disease exacerbation; these include streptococcal infections, stress, smoking, and certain drugs, such as lithium and beta-blockers.[29–31]

Pathogenesis

Psoriasis is the result of a complex cutaneous immune reaction with a major inflammatory component involving elements of the innate and adaptive immune systems and abnormal keratinocyte proliferation and differentiation. Activation of antigen-presenting cells leads to the preferential development of Th1- and Th17-type T cells that migrate into and proliferate within the skin. Homing mechanisms involve a variety of surface receptors and chemotactic factors, such as IL-8 and the cutaneous T-cell-attracting cytokine (CCL27). Several mediators have been identified that orchestrate many of the changes typical of psoriasis, including IL-12 and IL-23, TNF- α , and interferon γ (IFN- γ). In addition to epidermal hyperparakeratosis, angiogenesis leading to capillary abnormalities in the upper dermis, and a lymphocytic infiltrate, the histopathological changes seen in psoriasis include a marked influx of neutrophils, which may form sterile abscesses in the epidermis (i.e., so-called Munro's microabscesses).

Clinical Features

Plaque-Type Psoriasis

Plaque-type psoriasis, which is the focus of these guidelines, is the most common clinical form of the disease, accounting for more than 80% of all cases. This variant is characterized by sharply demarcated erythematous and scaly

plaques, typically located on the extensor surfaces of the extremities. Lesions may remain stable for long periods or progress to involve larger areas of the body.

Guttate Psoriasis

Guttate psoriasis presents with small, widely distributed erythematous papules with mild scaling. It is often the first clinical manifestation of psoriasis, particularly when triggered by a streptococcal infection. A later transition to plaque psoriasis may occur.

Intertriginous Psoriasis

Intertriginous psoriasis is defined by plaques located predominantly or exclusively in major skin folds, such as the axillae, abdominal folds, submammary area, and inguinal/gluteal clefts.

Inverse Psoriasis

Inverse psoriasis is characterized by plaques primarily affecting flexural areas without involvement of the typical predilection sites (e.g., extensor surfaces). It often overlaps with intertriginous psoriasis in clinical presentation.

Pustular Psoriasis

Pustular psoriasis manifests in several clinical subtypes. Generalized pustular psoriasis (also known as von Zumbusch psoriasis) is characterized by the widespread appearance of initially scattered, then confluent pustules, often accompanied by fever and generalized lymphadenopathy.

Palmoplantar Pustulosis

Palmoplantar pustulosis is a genetically distinct condition that may represent a separate disease entity. It is characterized by the presence of fresh yellow or older brownish pustules that occur exclusively on the palms and/or soles.

Acrodermatitis Continua Suppurativa (Hallopeau)

This rare variant of pustular psoriasis is marked by pustules with severe inflammation at the tips of the fingers and/or toes, often rapidly leading to nail matrix damage and nail loss. Over time, destruction of the distal phalanges may occur.

Diagnostic Approach

The diagnosis of psoriasis vulgaris is primarily based on the clinical appearance of skin lesions. Auspitz's sign—multiple fine bleeding points observed when psoriatic scale is removed—may be present in scaly plaques. Involvement of typical predilection sites and the presence of nail changes support the diagnosis.

Psoriasis can sometimes be difficult to differentiate from nummular eczema, tinea, or cutaneous lupus. Guttate psoriasis may resemble pityriasis rosea, and mycosis fungoides should be excluded in atypical cases. For intertriginous lesions, differential diagnoses include intertrigo and candidiasis. In uncertain cases, histopathological examination of a skin biopsy taken from the edge of a representative lesion may be required to confirm the diagnosis.

Severity Assessment

Several tools are available for assessing the severity of plaque psoriasis. The most widely used is the Psoriasis Area and Severity Index (PASI).